



RUN DATE: [REDACTED]
RUN TIME: [REDACTED]
RUN USER: [REDACTED]

PATIENT: [REDACTED]

ACCT #: [REDACTED]

LOC: [REDACTED]

AGE/SX: [REDACTED]

ROOM: [REDACTED]

REG DR: [REDACTED]

DOB: [REDACTED]

BED: [REDACTED]

STATUS: [REDACTED]

SPEC #:

RECD:

STATUS:

PERFORMED AT [REDACTED]

COLL:

TIME IN FORMALIN:

hrs.

COLD ISCHEMIA TIME:

mins.

CLINICAL INFORMATION:

Pre-Op Diagnosis:

Remarks:

Specimen(s): A. Right breast lumpectomy
B. 1st sentinel lymph node
C. 2nd sentinel lymph node

ICD.O-3

Carcinoma, infiltrating duct NOS
8500.3

Site @ Breast NOS
C50.9

W 4/14/14

MICROSCOPIC DIAGNOSIS

A. RIGHT BREAST LUMPECTOMY:

- INVASIVE DUCTAL CARCINOMA
- SEE COMMENT FOR DETAILS

B. FIRST SENTINEL LYMPH NODE:

- TWO REACTIVE LYMPH NODES

C. SECOND SENTINEL LYMPH NODE:

- ONE REACTIVE LYMPH NODE

COMMENT(S)

COLLEGE OF AMERICAN PATHOLOGISTS' PROTOCOL FOR EXAMINATION OF SPECIMENS WITH INVASIVE CARCINOMA OF THE BREAST, BASED ON AJCC/UICC TNM, 7TH EDITION
The following classification should be adjusted based on additional clinical information.

PROCEDURE:

Right breast lumpectomy and sentinel lymph node biopsies

LYMPH NODE SAMPLING:

Sentinel lymph nodes

SPECIMEN LATERALITY:

Right

HISTOLOGIC TYPE:

Invasive ductal carcinoma

TUMOR SIZE:

4 cm

HISTOLOGIC GRADE:

Tubular differentiation: Score 3

Nuclear pleomorphism: Score 3

Mitotic rate: Score 3

Overall grade: Grade 3

TUMOR FOCALITY:

Single focus of invasive carcinoma

DUCTAL CARCINOMA IN SITU:

Indeterminate

EXTENT OF TUMOR:

Skin: Uninvolved by invasive carcinoma

MARGINS:

Margins uninvolved by invasive carcinoma

** CONTINUED ON NEXT PAGE **

RUN DATE:

RUN TIME:

RUN USER:

Specimen Inquiry

Lab Database:

PAGE 2

SPEC #:

PATIENT:

(Continued)

COMMENT(S)

(Continued)

Distance from closest margin: 6 mm,
deep/posterior

LYMPH NODES:

Number of sentinel lymph nodes examined: 3
Total number of lymph nodes examined: 3
Number of lymph nodes with macrometastasis: 0
Number of lymph nodes with micrometastasis: 0
Number of lymph nodes with isolated tumor cells: 0
Number of lymph nodes without tumor cells
identified: 3

PATHOLOGIC STAGING:

Primary tumor: pT2
Regional lymph nodes: pN0 (i-)
Distant metastasis: Not applicable

GROSS DESCRIPTION:

A. Received fresh in the operating room labeled with the patient's name and "right breast lumpectomy" with request for gross evaluation of margins as well as selection of tissue for tissue banking is a lumpectomy specimen which weighs 131 grams and measures 9 x 8 x 5 cm. It is partially covered by a 9 x 1.5 cm ellipse of dark brown skin. Two sutures orient the specimen as follows: long suture is 10:30 and short suture is 4:30. A palpable mass is present within the specimen. The tissue surrounding the mass moves easily. The specimen is inked as follows: 10:30-4:30 blue, 4:30-10:30 black, 4:30 edge red, 10:30 edge yellow, and deep/posterior green. The specimen is serially sectioned revealing a fairly well circumscribed grayish-tan mass with hemorrhagic needle tracts which measures 4 cm in length, 3 cm in depth, and 3 cm in width. Towards the 4:30 aspect, the tumor has a somewhat more lobulated appearance. The margins are negative with the closest margin deep which measures 1.5 cm. A portion of the tumor is provided to the tissue bank coordinator. The specimen is placed in formalin with sections to be held overnight for optimum formalin fixation.

- A1 - skin and underlying breast tissue
- A2-A3 - the tumor as it most closely approximates to green, inked margin
- A4 - tumor as it most closely approximates to blue, inked margin
- A5 - additional representative section of tumor
- A6 - tumor grossly extending into surrounding ducts
- A7 - section of fibrotic area near the red, inked margin

B. In formalin, labeled with the patient's name and "first sentinel lymph node" is a 2.5 x 2 x 1 cm ovoid tan lymph node covered by a small amount of adipose tissue. On cross section, the node is largely composed of adipose tissue. It is totally submitted in blocks B1 through B3 per sentinel lymph node protocol. The specimen was evaluated late in the day and is found to be unfixed. It is held for overnight fixation prior to tissue processing.

C. In formalin, labeled with the patient's name and "second sentinel lymph node" is a 1.5 x 1.5 x 1 cm ovoid portion of fatty tissue which contains a 1.5 x 1 x 0.5 cm fatty lymph node. The lymph node is submitted in block C1 to be held for overnight formalin fixation, prior to further tissue processing.

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RUN DATE:
RUN TIME:
RUN USER:

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SPEC #: PATIENT: (Continued)

MICROSCOPIC DESCRIPTION:

The tumor is a well delineated, nodular, necrotic, poorly differentiated carcinoma with bizarre nuclei, increased mitotic activity. There is very little tubular differentiation. Clinical and radiographic correlation is recommended.

INTRAOPERATIVE CONSULTATION:

RIGHT BREAST LUMPECTOMY:

- GROSS TUMOR PRESENT WITH CLOSEST MARGIN DEEP MEASURING 1.5 CM
- TUMOR TISSUE PROVIDED TO TISSUE BANK COORDINATOR
- RESULTS GIVEN TO DR. IN OPERATING ROOM AT

PHOTO DOCUMENTATION

Image - Picture Copy Error

Signed ____ (signature on file) ____

** END OF REPORT **

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HPA Discrepancy		✓
Prior Discrepancy History		✓
Dual/Synchronous Primary Note		✓
Case is (c)le		
Reviewer Initials	kmf	
Date Reviewed	11/13	

CONFIRMED / DISQUALIFIED